
AFTER THE DEMO • FOR YOUR REVIEW

What you saw, and what we'd recommend.

A short, honest record of the demo — what it did on your own files, what it costs your staff in real terms, what happens when it doesn't work perfectly, and the smallest first step.

PREPARED FOR — TYPE A PRACTICE

Practices with no digital system today.

Nothing digital yet. A back room of paper. No view of the practice.

This document is written specifically for a practice in your situation. It is not a generic brochure — the recommendation in it is the one configuration we would actually put in your practice.

READ THIS
BEFORE YOU DECIDE.
IT IS DELIBERATELY SHORT.

You run on paper. In the demo, we took a stack of your own files — the disorganised ones, the handwritten ones — and showed you what comes out the other side: structured, searchable patient records, with the original scan one click away.

— Your own files, read automatically

Not a prepared sample. Your handwriting, your forms, your medical-aid stamps — extracted into structured fields while you watched.

— A question you can't answer today, answered

We asked the system a plain-English question across the records — the kind of question that today means an afternoon in the back room — and it returned the answer with the source page attached.

— Nothing on your desk changed

No new screen for you to learn during a consult. The work happens around you, not in front of you.

02 WHAT THIS ACTUALLY COSTS YOU IN STAFF TIME

The honest answer to the question you are really asking — “my reception is already drowning, what does this add to her day?”

<i>Per document</i>	30–60 seconds. The system extracts; your staff reviews what it found, corrects anything wrong, and approves. They know your handwriting; the system handles most of it; they catch the rest.
<i>Week one</i>	Scanning happens into the same folder your reception already scans into. The only new habit is the validation queue — a list of documents waiting for a 30-second check. That is the entire workflow change.
<i>Month three</i>	The queue is a background habit, not an event. The structured archive is large enough that the search and analytics start answering real questions. Nothing about the consult has changed.
<i>What does not change</i>	How you consult. What you write. Your existing system, if you have one. We are a layer that adds capability, not a system that takes work away from one that already works.

03 WHAT HAPPENS WHEN IT DOESN'T WORK PERFECTLY

Demos show the happy path. This is the part that matters more: what the system does when a scan is bad, the handwriting is unreadable, or the extraction is unsure. We would rather you read this now than discover it later.

Low-confidence extraction

Every extracted field carries a confidence score. Low-confidence fields are flagged for the validator, not silently accepted. The human sees exactly what the system was unsure about.

An unreadable document

If a scan is too poor to extract, it does not get guessed at. It is held in the queue, flagged, and a person decides — rescan, enter by hand, or set aside. The system never invents data to fill a gap.

The wrong-patient safeguard

Nothing enters a patient's record until a person confirms it is the right patient. The match is shown; the human says yes or no. Wrong-patient errors are caught at that gate — before they happen, not discovered months later.

Every action is logged

Every approval, correction and reversal is recorded — who did it, when. If a medical aid audits the practice, or you need to show the basis of a record, the history is one query away.

04 WHY A SOUTH AFRICAN PRACTICE CHOOSES THIS

BUILT HERE, NOT BOLTED ON

Medical-aid schemes, HPCSA prescription rules, NAPPI codes, the SA edition of ICD-10, local payments — native to the system, not adapted to it after the fact.

IT WORKS WITH WHAT YOU ALREADY HAVE

Export and ingest via FHIR, CSV or API. You do not rip out a system that works to get the parts you don't have.

DIGITISATION AS A SUBSCRIPTION, NOT A PROJECT

The reason practices stay on paper is the six-figure project quote nobody signs. A predictable monthly page allowance instead — payable, cancellable, no capital sign-off.

EACH PRACTICE'S DATA IS ISOLATED

Multi-tenant by design. What happens at one practice never crosses to another.

You have no digital system to protect or work around, which makes this the cleanest possible starting point. We'd move your core workflow onto one platform from day one, and digitise the back room in parallel.

The practice platform — Essential

Registration, encounter, prescription, invoice — one system from day one. Patient registry, vitals with auto-BMI, allergy-interaction checks on prescriptions, billing with online payments. The whole core loop, not a patchwork.

Digitisation of the back room

Your legacy archive becomes structured patient data, month by month, at a fixed monthly pace. By the time a 2014 patient walks back in, their history is already in the system.

The analytics view

Once records are structured, the view of your practice you have never had — chronic-disease cohorts, who is overdue, where revenue is leaking — without you building a single spreadsheet.

What we would *not* start you on: AI Scribe, telehealth, or the Clinical AI beta. Those are later conversations, if ever. The first job is getting your practice off paper without disrupting a single consult.

The same discipline we apply to the product, applied to this document.
These are real and they are not in scope of what you would be buying.

FINAL PRICING

Set per practice on doctor count, page volume and integration scope. The pilot price is fixed and stated; the full quote is a conversation, not a number on a leave-behind.

CLINICAL DECISION SUPPORT

Our clinical-AI work is an invite-only beta, not a commercial product, with no medical-device approval. It is a design-partner conversation if you raise it — it is deliberately not part of what this document offers you.

ROADMAP ITEMS

Medical-aid claims auto-submission, multi-site management and native EHR integrations are planned, not promised here. If it is not in the configuration above, treat it as not yet real.

THE SMALLEST FIRST STEP

One month. Your files. No lock-in.

Not the whole archive. Not a year's commitment. One month, 1,500 pages, the files you choose — ideally the messiest ones, so you are testing the hard case, not the easy one. Your staff validates as they would in production. At the end of the month you have a real, structured slice of your own archive and an honest sense of the day-to-day. Cancel any time, no setup fee, no lock-in.

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